EXECUTIVE SUMMARY | DECISION REQUIRED TODAY

# PAUSE.

## Do not sign yet. Prove it in 12 weeks. Then decide.

**0 of 8**

### We have no proof about the target.

None of the eight executives interviewed cited a single verified fact about it.

**5 gates**

### India can delay or block this deal.

FDI route | Press Note 3 | CCI clearance | licences | data law

**?**

### The real price is unknown.

Remediation, integration and retention costs are not yet costed.

**48%**

### The market is real. The target is unproven.

Government now funds 48% of India's health spend: do not walk away.

## DECIDE TODAY

| 1 | 2 | 3 | 4 |
|---|---|---|---|
| NO BINDING OFFER | FUND 12-WEEK DILIGENCE | REGULATORY MAP IN 2 WEEKS | SET HURDLE RATE AND MAX PRICE |

| WEEK 2 | WEEKS 2-10 | WEEK 12 \| INVESTMENT COMMITTEE |
|---|---|---|
| Regulatory kill gate | Licence, commercial, financial, data and people gates | GO \| RE-PRICE \| WALK AWAY |

## BOTTOM LINE

**Buying now means paying a premium for an advantage nobody has proven. Pausing costs 12 weeks and keeps the deal alive.**

Sources: SRC-01 revised stakeholder transcripts (8 interviews, 48 verbatim statements); EXT-01 PIB/MoHFW NHA 2021-22; EXT-03/05/07/09/11/13 Indian rules. Detail on pages 2-8.

THE DECISION

# Pausing with gates is the only option that is reversible, evidence-led and answers 'why buy rather than partner?'

| Option | Fit to evidence | Reversibility / risk | Answers 'why acquisition?' | Verdict |
|---|---|---|---|---|
| **A Proceed now** | Does not meet | Does not meet | Does not meet | Not recommended: zero target-specific evidence; Indian approvals unmapped; four stakeholders set explicit pause conditions. |
| **B Pause pending validation (recommended)** | Meets | Meets | Partially meets | Recommended: preserves the option, tests the thesis, clears Indian gates before capital is committed. |
| **C Reconsider: partnership or organic entry** | Partially meets | Meets | Unknown | Keep live as the benchmark inside Option B (required by S01-06); do not pick it yet without evidence. |
| **D Walk away** | Does not meet | Meets | n/a | Premature: no evidence disproves the thesis and the India context is supportive. |

Qualitative appraisal: no cost or return data exists to score options numerically, so no weighted scores are asserted.

## Why not proceed, and why not walk away

| Why not proceed now | Why not walk away |
|---|---|
| The thesis is **speed** (CSO), but the team has not shown which advantages the target owns or whether a partner could deliver them (S01-03). Indian approvals are unmapped. Remediation, integration and retention are uncosted. Four executives have set explicit pause conditions (S01-06, S03-04, S04-05, S06-05). | Nothing in the evidence disproves the thesis. India's government health spending rose from 1.13% to 1.84% of GDP (2014-15 to 2021-22); PM-JAY has funded 13.25 crore admissions worth INR 2.03 lakh crore. Pausing keeps the option open at low cost. |

## What would change our recommendation

| If diligence shows that... | We would recommend... |
|---|---|
| The target holds exclusive licences, contracts or relationships a partner cannot provide, and Indian approvals are routine | **Proceed**, with price discipline and SPA protections |
| The advantage is not exclusive, or approvals carry thesis-breaking conditions | **Reconsider**: partnership or organic entry |
| Material unresolved safety findings, unclear licences, bribery evidence or undisclosed regulator correspondence | **Stop**, or re-price with escrow, specific indemnities and conditions precedent |
| The downside case fails the client's hurdle rate | **Re-price or walk away** |

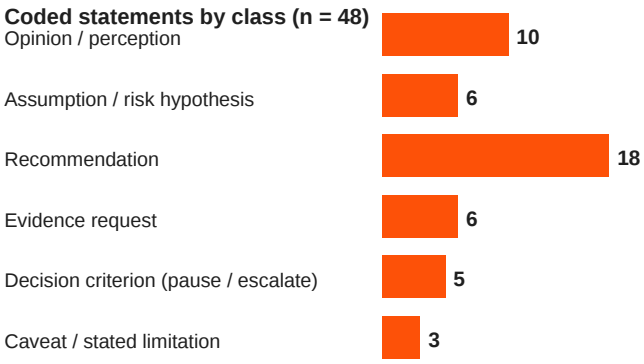
## Decisions requested today

**1.** Do not sign a binding offer on current evidence.
**2.** Approve a 12-week, gated confirmatory diligence with explicit kill criteria (G0-G5).
**3.** Mandate an India regulatory route map in the first two weeks as the first kill gate.
**4.** Supply the investment thresholds (hurdle rate, maximum price, risk appetite) the downside case must clear.
**5.** Keep partnership / organic entry live as the benchmark the acquisition must beat.

Source: SRC-02 decision options; SRC-01 statement IDs; EXT-01, EXT-29.

WHAT THE EVIDENCE SAYS

# Eight senior voices agree the case is untested; verified Indian data shows a large, increasingly publicly funded market




| Theme | Voices (of 8) |
|---|---|
| T1 Strategic rationale and alternatives | 4 |
| T2 Market access and adoption reality | 3 |
| T3 Licence to operate: clinical quality and regulation | 4 |
| T4 Health data, cyber and technology integration | 1 |
| T5 Financial quality and true cost of the deal | 5 |
| T6 Operating capacity and Day-1 continuity | 4 |

**Reported fact about the target: 0**

## India sector intelligence (verified)

| Area | Verified signal | Source | So what for the deal |
|---|---|---|---|
| Pharma | USD 60 bn domestic market; USD 130 bn projected by 2030; exports USD 30.5 bn | EXT-27 | Scale and export pull; intense generic price competition |
| Medical devices | USD 11 bn (2020); policy target USD 50 bn; 21 PLI projects making 54 devices | EXT-28, EXT-31 | Import substitution; domestic rivals scaling up |
| Public payer | Govt = 48.0% of total health spend (2021-22); PM-JAY 13.25 crore admissions, INR 2.03 lakh crore | EXT-01, EXT-29 | Volume via schemes; package-rate price pressure |
| Seniors (70+) | Vay Vandana: 23.46 lakh admissions, INR 5,668.80 crore in two years | EXT-32 | New demand pool: cataract, dialysis, cardiac |
| Digital health | 93 crore+ ABHA; 104 crore+ linked records | EXT-30 | Interoperability expected; data-law exposure |
| Policy scrutiny | Hospital price-display enforcement (Oct 2026); hospital FDI relook urged (partial) | EXT-33, EXT-34 | Margin and ownership-policy risk for providers |

## Competitive pressure by business model

| If the target is... | Pressure (verified signal) | Value implication |
|---|---|---|
| Pharma (generics / branded) | Price ceilings under DPCO; large domestic manufacturing base; export-led scale | Pricing upside capped by price control and generic competition |
| Medical devices | PLI-backed domestic makers entering high-end devices (MRI, CT, LINAC, stents) | Import-led incumbents face local substitution |
| Hospitals / diagnostics | Scheme package rates; price-display enforcement; reported FDI relook | Pricing power constrained; ownership policy uncertain |
| Health-tech | ABDM public rails; consent-based data sharing | Data moats weaker; compliance heavier |

Named competitors are deliberately not listed: the target's segment is not defined in the case (SRC-01, Section 6). Gate G2 builds a share-by-product map once confirmed. Sources: EXT-01, EXT-18, EXT-27 to EXT-34 (EXT-34 partial).

FIVE INSIGHTS FOR THE DECISION

# Each insight is anchored to verbatim stakeholder statements and, where possible, Tier 1 Indian sources

**1**

### The deal thesis is 'speed', but nobody has yet shown the target owns the advantages that create speed

**Evidence:** The strategic hypothesis is plausible but untested. Zero of eight stakeholders cites target-specific evidence; every interview is framed as conditions and evidence requests.

**Why it matters:** If the advantage is not exclusive, the premium paid for speed buys nothing a partnership could not deliver at lower risk and lower capital. **Next:** Build a target-asset inventory (licences, contracts, relationships, distribution); test exclusivity and transferability; run a partnership/organic comparison.

*Confidence: High that the thesis is unvalidated; unknown whether it is true. Anchors: S01-01, S01-03, S01-06, S06-05*

**2**

### India's health market is large and growing, but government is now the biggest payer: big is not the same as accessible

**Evidence:** Verified Indian data supports their caution. Government's share of total health expenditure rose to 48.0% in 2021-22 and out-of-pocket fell to 39.4% (EXT-01). PM-JAY alone has funded 13.25 crore admissions worth INR 2.03 lakh crore (EXT-29). Public purchasing, package rates and price control (DPCO) shape adoption and price.

**Why it matters:** Revenue forecasts that assume private-market pricing or easy cross-sell will overstate value if the target depends on public payers, tenders or price-controlled products. **Next:** Map the decision chain; quantify payer mix (PM-JAY/CGHS/tender/private), DPCO exposure, change-of-control clauses and customer retention via reference calls.

*Confidence: High for the macro shift (Tier 1 data); low for target-level impact. Anchors: S01-04, S02-02, S08-01, S08-05*

**3**

### In India, 'licence to operate' is a gate on the deal, not a line in the price

**Evidence:** Indian rules make several of these binary: brownfield pharma above 74% foreign ownership needs Government approval with five-year NLEM-production and R&D; conditions (EXT-03); land-border beneficial ownership forces the Government route (EXT-05); deals above INR 2,000 crore with substantial Indian operations need prior CCI approval, with a 30-day prima facie view and 150-day outer limit (EXT-07/08/09); a >50% voting change under MDR 2017 triggers a fresh device licence application (EXT-11); Schedule M GMP is binding (EXT-12); companies are liable for bribery by associated persons (PCA s.9, EXT-22).

**Why it matters:** Any of these can delay closing, impose conditions that change the business case, or stop the deal. They must be cleared before signing, not discovered after. **Next:** Commission an India regulatory route map (FEMA/FDI, Press Note 3, CCI, SEBI SAST if listed, CDSCO/state licences, CEA, UCPMP, PCA) and an independent quality/inspection review.

*Confidence: High for the rules (Tier 1 sources); applicability unconfirmed. Anchors: S04-02, S04-05, S03-04, S01-07*

**4**

### The headline price understates the true cost: remediation, integration and retention are not yet costed

**Evidence:** There is broad agreement on what to cost and zero data on how much. Indian compliance obligations (Schedule M in force; DPDP substantive duties from 13 May 2027, computed; Labour Codes in force since 21 Nov 2025) make some remediation non-optional.

**Why it matters:** Returns are decided by total cost of ownership. Uncosted remediation is a common way an attractive acquisition becomes value-destructive. **Next:** Independent QoE; IVS-compliant valuation; tax diligence; revenue-to-cash reconciliation; costed remediation and integration register; downside case against client hurdle rates.

*Confidence: High that costs exist; magnitude unknown. Anchors: S06-02, S06-03, S05-06, S01-05*

**5**

### Integration is a patient-safety and data-law event: Day 1 must protect continuity of care and health data

**Evidence:** Indian law adds hard clocks: CERT-In requires specified cyber incidents to be reported within 6 hours (EXT-16); DPDP requires breach intimation to the Board and each affected person, with penalties up to INR 250 crore (EXT-13, EXT-15). ABDM's 93 crore+ health accounts raise the interoperability bar (EXT-30).

**Why it matters:** A disrupted service or a data breach during integration destroys trust and value faster than any synergy can create it. **Next:** Critical-operations and system-criticality map; incident and breach history; DPDP readiness; NIST CSF 2.0-based cyber review; Day-1 continuity plan and TSA with a named stop authority.

*Confidence: High that the risk is real; exposure unknown. Anchors: S07-04, S03-02, S05-04, S07-05*

Sources: SRC-01; EXT-01/03/05/07/09/11/12/13/14/15/16/18/21/22/24/26/29/30.

INDIAN RULES AND REGULATIONS

# Licence to operate is a gate on the deal, not a line in the price

| Rule-set (authority) | What it requires | Applies when | Deal impact | Status |
|---|---|---|---|---|
| FDI Policy / FEMA NDI Rules (DPIIT) | Pharma greenfield 100% automatic; brownfield up to 74% automatic, above 74% Government approval; NLEM production and R&D; spend held for 5 years; non-compete not permitted. Medical devices 100% automatic. | Foreign acquirer | Approval time; 5-year operating conditions | Verified EXT-03/04 |
| Press Note 3 (2020) | Investor or beneficial owner from a land-border country: Government route only; later transfers into that category also need approval. | Any such owner in the chain | Approval in any sector | Verified EXT-05; 2026 changes partial |
| Competition Act 2002, as amended 2023 (CCI) | Prior approval where deal value > INR 2,000 crore and substantial Indian operations (from 10 Sep 2024), plus asset/turnover tests; 30-day prima facie view; 150-day outer limit. | Thresholds met | Standstill until clearance | Verified EXT-07/08/09 |
| Drugs & Cosmetics Act; revised Schedule M | Revised GMP: large makers from 28 Jun 2024; small/medium makers (Form A) to 31 Dec 2025. | Drug manufacturer | Remediation capex; licence risk | Verified EXT-12 |
| Medical Devices Rules 2017 (CDSCO) | >50% voting change = change in constitution; fresh licence application within 180 days. | Device maker / importer | Licence continuity | Partial EXT-10/11 |
| DPDP Act 2023 + Rules 2025 (MeitY); CERT-In 2022 | Consent notices; breach intimation; penalties up to INR 250 crore; most duties from 13 May 2027 (computed). Cyber incidents reported within 6 hours; logs kept 180 days. | Always (health data) | Compliance cost; Day-1 security | Verified; dates computed |
| Prevention of Corruption Act s.9 (2018) | Company liable when an associated person bribes a public servant; defence: adequate procedures. | Always | Inherited liability | Verified EXT-22 |
| Four Labour Codes (in force 21 Nov 2025) | Replace 29 labour laws: wages, industrial relations, social security, OSH. | Always | People cost; HR compliance | Verified EXT-21 |
| SEBI SAST 2011 | 25% voting rights or control triggers an open offer (minimum 26%). | Target listed | Price and cash | Verified EXT-17 |
| DPCO 2013; UCPMP 2024; Clinical Establishments Act 2010 | Ceiling prices; ethical marketing code with disclosures; provider registration in 12 States + 7 UTs. | By business model | Price cap; conduct; licensing | Verified EXT-18/19/20 |

**Implication for the C-suite:** which rules apply cannot be confirmed until the target's business model, ownership chain and deal value are known. That is why the regulatory route map is the first kill gate (G0), not a closing condition. A reported parliamentary relook at hospital FDI (EXT-34, partial) is a policy risk to monitor.

All sources accessed 31 Oct 2026. 'Partial' items must be confirmed by Indian legal counsel against Gazette text. Not legal advice.

DILIGENCE AGAINST INTERNATIONAL M&A; STANDARDS

# Benchmarked against recognised international practice: 8 gaps in v1 are now closed as workstreams

| # | Workstream | International / Indian reference | v1 | v1.1 | Gate |
|---|---|---|---|---|---|
| 1 | Strategic rationale and alternatives | Investment-committee practice | Covered | Covered | G0, G5 |
| 2 | Commercial (market, customers, competitors) | Commercial due diligence practice | Partial - no sector scan | Sector intelligence and competition map added | G2 |
| 3 | Financial / quality of earnings | QoE practice; Ind AS 103 / IFRS 3 purchase-price allocation after close | Covered | PPA step added | G3 |
| 4 | Valuation | IVS effective 31 Jan 2025 (IVSC); FEMA pricing rules (counsel) | Not covered | Added | G3 |
| 5 | Tax | Tax diligence practice (direct, indirect, transfer pricing) | Not covered | Added as workstream | G3 |
| 6 | Legal and licence to operate | Indian sector law: FDI, MDR, Schedule M, CEA, DPCO, UCPMP | Covered | Corrected and upgraded | G0, G1 |
| 7 | Merger control | Competition Act 2002 as amended 2023 (CCI) | Covered | Tier 1 source; 30-day / 150-day | G0 |
| 8 | Anti-bribery, corruption, sanctions | PCA 1988 s.9; FCPA / UK Bribery Act if nexus; DOJ M&A; Safe Harbor (2023); OECD Ch. VII | Partial (UCPMP only) | Added | G1 |
| 9 | People and labour | Four Labour Codes (21 Nov 2025); key-person retention | Partial | Added | G3, G4 |
| 10 | Data privacy and cyber | DPDP 2023 + Rules 2025; CERT-In 2022; NIST CSF 2.0 | India rules covered | International framework added | G4 |
| 11 | Clinical quality and patient safety | Schedule M; MDR 2017; CEA 2010; independent clinical review | Covered | Covered | G1 |
| 12 | ESG / responsible business conduct | OECD Guidelines 2023: risk-based due diligence | Not covered | Added | G1, G4 |
| 13 | Deal protection (SPA) | Practice: conditions precedent, specific indemnities, escrow, W&I; insurance, MAC | Not covered | Added | G5 |
| 14 | Integration and Day 1 | Practice: 100-day plan, TSA, stability list, stop authority | Covered | TSA added | G4 |
| 15 | Evidence and AI governance | SRC-02 rubric and rules | Covered | Covered | All |

Sources: EXT-09 CCI; EXT-21 PIB; EXT-22 PCA; EXT-23 DOJ (archived page; confirm status if US nexus); EXT-24 IVSC; EXT-25 OECD; EXT-26 NIST. Conformance is our assessment, not a certification: no single certifiable international M&A; standard exists.

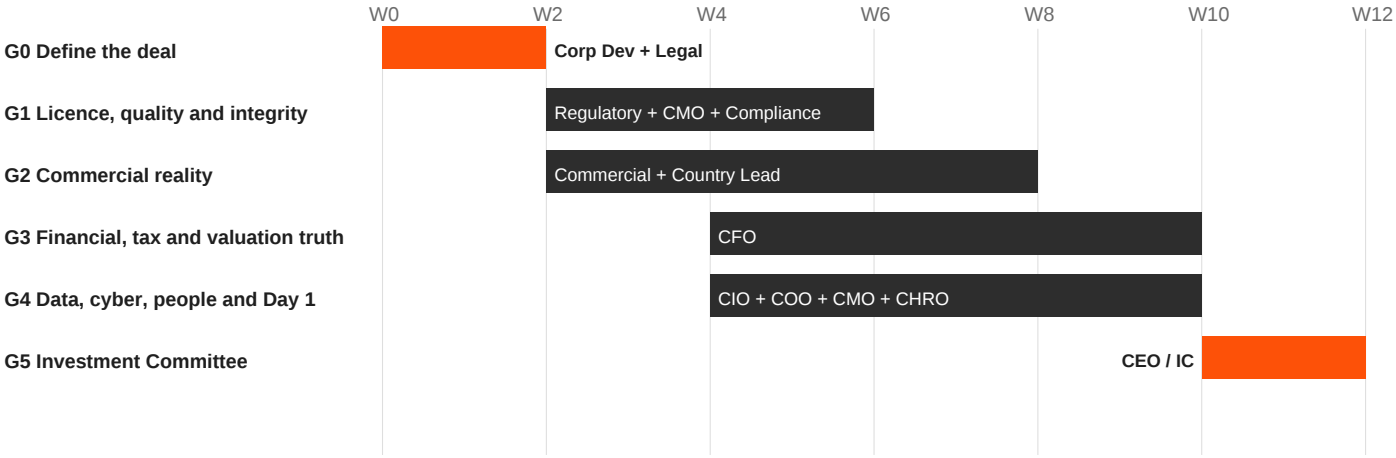
## Risk register

| ID | Risk | Impact if true | Testable before signing? | Gate |
|---|---|---|---|---|
| R1 | Advantage not exclusive; a partner could deliver it | High | Yes | G0, G5 |
| R2 | Approval route adds conditions or delay (FDI, PN3, CCI) | High | Yes | G0 |
| R3 | Licence or quality failure (Schedule M, MDR, CEA) | High | Yes | G1 |
| R4 | Inherited bribery or improper HCP conduct (PCA s.9, UCPMP) | High | Partly | G1 |
| R5 | Revenue tied to schemes / price ceilings; weak cash conversion | High | Yes | G2, G3 |
| R6 | Uncosted remediation, integration, retention | High | Yes | G3 |
| R7 | Health-data breach or non-compliance (DPDP, CERT-In) | High | Partly | G4 |
| R8 | Care or supply disruption during integration | High | Partly | G4 |
| R9 | Key-person loss; Labour Code cost impact | Medium | Yes | G3, G4 |
| R10 | Policy shift: hospital FDI relook; price transparency | Medium | No - monitor | G2 |

Impact is analyst judgement; likelihood is not rated because no target data exists yet.

VALIDATION ROADMAP AND NEXT STEPS

# Twelve weeks, six gates: regulatory first, Investment Committee last




Durations and owners are proposals for client confirmation. Sign-to-close is separate: if notifiable, CCI review (30-day prima facie; 150-day outer limit) and any FDI approval run after signing.

## Kill criteria

| Gate | Scope | Stop or re-price if |
|---|---|---|
| G0 Define the deal | Confirm target business model, Indian entities, ownership chain, deal value and structure; produce the India regulatory route map. | Government route needed with conditions (e.g., NLEM production) that break the thesis; or timeline incompatible with strategy. |
| G1 Licence, quality and integrity | Licence register; inspection, warning and recall history; Schedule M / MDR compliance; UCPMP; CEA/state registration if a provider; independent clinical review; anti-bribery and sanctions screen; ESG / responsible-conduct screen. | Unresolved material safety finding, unclear licence, evidence of bribery, or uncosted remediation (S03-04, S04-05). |
| G2 Commercial reality | Decision-chain map; customer reference calls; change-of-control clauses; payer/scheme mix; DPCO price exposure; competitor map; key-person dependency. | Key contracts terminable on change of control; advantage not exclusive (S01-07). |
| G3 Financial, tax and valuation truth | Independent QoE; revenue-to-cash; tax diligence; IVS-compliant valuation; costed remediation, integration and retention register; Labour Code impact; downside case vs client thresholds. | Downside case fails the client's hurdle rate (to be supplied). |
| G4 Data, cyber, people and Day 1 | Data inventory and consent basis; DPDP readiness; CERT-In incident history; NIST CSF 2.0 review; system criticality map; Day-1 continuity plan, TSA and stop authority; retention plan. | Unremediated material breach, unknown health-data flows, or no credible continuity plan. |
| G5 Investment Committee | Acquisition vs partnership vs organic; consolidated gate results; SPA protections (conditions precedent, specific indemnities, escrow, W&I;); go / re-price / no-go. | Acquisition not superior to the alternative (S01-06). |

## Open questions for the client

1. What is the target's business model (pharma, devices, provider, diagnostics or health-tech) and which Indian states does it operate in?
2. Is the target listed? What is the expected deal value and the acquirer's ownership chain?
3. Does any beneficial owner on either side sit in a country sharing a land border with India?
4. What share of revenue comes from public payers, schemes, tenders or price-controlled products?
5. What licences, inspections, warnings, recalls or regulator correspondence exist in the last three to five years?
6. What health data does the target hold, where is it hosted, and has it had reportable incidents?
7. What hurdle rate and maximum price will the executive committee accept?
8. What would a partnership deliver, at what cost and speed, compared with buying?

FINAL CHECKS, CORRECTIONS AND AI GOVERNANCE

# Every headline claim passes the four investor tests, or is labelled where it only partly passes

| Claim | Where from? | Verifiable? | Trustworthy? | Why it matters |
|---|---|---|---|---|
| Acquisition rationale is speed | S01-01 (CSO transcript) | Yes - verbatim in SRC-01 | Fictional case input; one stakeholder's opinion | It is the premium we would pay; if unproven, the price is unjustified |
| No stakeholder cites target-specific evidence | Full read of SRC-01, Interviews 01-08 | Yes - all 48 coded statements reviewed | Analyst coding, reproducible | The decision cannot rest on evidence that does not exist yet |
| Government is 48.0% of India's total health spend (2021-22) | EXT-01 PIB; EXT-02 NHSRC | Yes - two official sources agree | Tier 1 government | Public payers shape price and adoption |
| Brownfield pharma >74% FDI needs Government approval; 5-year NLEM/R&D; conditions | EXT-03 PIB; DIPP note via ET | Yes | Tier 1 + Tier 2 | Can delay closing and constrain the operating plan |
| CCI: > INR 2,000 crore with substantial Indian operations; 30-day / 150-day | EXT-07/08; EXT-09 CCI | Yes - regulator and two independent sources | Tier 1 + Tier 2 | Standstill: cannot close before clearance |
| Company liable for bribery by associated persons (PCA s.9) | EXT-22 statute text | Yes | Tier 1 statute | Inherited misconduct becomes the acquirer's problem |
| MDR 2017: >50% voting change triggers fresh device licence | EXT-11 secondary | Partially - consultant summary | Tier 3; confirm with Gazette | Could interrupt device sales if not filed on time |
| DPDP substantive duties from 13 May 2027 | EXT-13 PIB; EXT-14 computed | Partially - 18 months confirmed; exact date computed | Tier 1 + secondary | Integration will likely run into the compliance date |
| CERT-In 6-hour incident reporting | EXT-16 CERT-In | Yes - official direction | Tier 1 regulator | Day-1 security operations must be ready |

## v1 to v1.1 corrections

| Item | v1 | v1.1 |
|---|---|---|
| Clinical Establishments Act coverage | '19 States/UTs (all UTs except Delhi)' | 19 = 12 States + 7 UTs (all UTs except NCT of Delhi); list checked on MoHFW portal |
| CCI review timeline | 150 days from a law-firm blog (partial) | Upgraded to Tier 1 (CCI): 30-day prima facie view; 150-day overall limit |
| Pharma brownfield FDI conditions | 'NLEM output and R&D; maintained' | Five-year period confirmed (DIPP press note via ET, 2016) |
| Hospital FDI route | 'Confirm (secondary: 100% auto)' | Kept partial; reported parliamentary relook (Aug 2026) added as policy risk |
| Press Note 3 (2026 changes) | One secondary source | Two secondary sources describe it differently; kept partial; counsel to confirm |
| Scope vs international practice | No ABC, labour, ESG, valuation, tax, SPA protections or sector scan | All added as workstreams with gates and owners |
| Document status | 'Draft for partner review' | v1.1 final for stakeholder circulation; version control added |

**Evidence base and governance:** SRC-01 revised transcripts and SRC-02 problem statement (both trainer-supplied 31 Oct 2026, used in full); SRC-03 Stage 1-6 method; SRC-04 prior AI outputs (method reused, content rejected). 34 external sources (26 Tier 1), accessed 31 Oct 2026; 4 external facts remain partially verified and are assigned to counsel in G0. All 48 quotes machine-checked against the transcript. AI was used as a productivity assistant; the recommendation is owned by the consulting team and requires partner sign-off. Full register, statement ledger and prompts are in the working-group document.